

RESMED INC RMD

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by

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Price:	228.00	EPS	0	0
Shares Out. (in M):	146	P/E	0	0
Market Cap (in \$M):	32,900	P/FCF	0	0
Net Debt (in \$M):	-1	EBIT	0	0
TEV (in \$M):	32,300	TEV/EBIT	0	0

Description

ResMed (Ticker: RMD) is a medical device company that makes machines and masks to treat sleep apnea and other breathing disorders.

ResMed grew revenues at 9.5% last year, and converted > 90% of its operating cash flow into free cash flow. You are buying a high-ROIC (25%) duopoly with an 8.2% FCF yield on enterprise value trading at an atypical discount (~16x EV/EBIT) because a crowded 10% short interest fundamentally misunderstands the clinical mechanics of GLP-1s and the recurring cash flow of mask resupply.

Clinical Reality and TAM

An estimated 30 million Americans suffer from obstructive sleep apnea (OSA), and worldwide that number approaches a billion. It is one of the most fatal sleep disorders because repeating cycles of airway collapse during sleep result in intermittent hypoxia, and sleep fragmentation.

Invented in 1980, continuous positive airway pressure (CPAP) treatment for OSA takes air from a room, filters it, and blows a steady stream of pressurized air through a tube into a mask sealed over the nose and mouth, keeping the airways open. The newest devices offer automated pressure adjustments (e.g., lighter pressure earlier in the sleep cycle), heated humidification (to counteract issues from being dried out), and wireless tracking.

The Short Thesis and Why We Question It

ResMed has been written up three times on VIC, all on the short side. Today, short interest stands at 10% of the float. The reason is obvious. When I mention ResMed as a long, I always hear the same thing: Aren't GLP-1s going to destroy their business? The core short thesis is that GLP-1s, while they do not cure severe OSA, do achieve symptom relief in approximately two-thirds of patients in the mild to moderate category, (based on losing 15-20% of their weight) while taking GLP-1s. The thesis is that these patients will abandon (or refuse to start) CPAP therapy. Let's examine this claim more carefully.

Take, for example, a February 2026 Komodo Health study that demonstrated an 83% decline in CPAP starts in OSA patients using Tirzepatide/Zepbound. We think it's easy to misinterpret the headline number: Among newly diagnosed OSA patients taking Tirzepatide, 4% initialized CPAP treatment, versus 22% of the control group. $((22\% - 4\%)/22\%) = 83\%$. Seems like strong evidence, but why did this occur? Was it because their OSA was resolved? Not necessarily.

GLP-1 patients are gradually ramped up on dosage, so real weight loss does not occur for 6-12 months. It's not like these patients lost 40 pounds in a week, their symptoms were resolved, and therefore they did not start CPAP. It may be more likely that physicians chose a "wait and see" approach before ordering CPAP equipment.

Additionally, the short side argues that because in the Komodo data 84% of Zepbound scripts were written by PCPs that this demonstrates that OSA can be resolved by addressing the root metabolic driver, obviating the need for secondary CPAP intervention.

We think this may relate to the fact that Zepbound received FDA approval for moderate to severe OSA. Commercial insurers routinely deny coverage for GLP-1s for general weight loss reasons, so now, doctors have a diagnostic code and a reimbursable way to pay for the injection. What the short side misinterprets as a clinical substitution is really a workaround to get insurance coverage for weight loss.

The short thesis relies on these kinds of data to argue that doctors treat OSA with GLP-1 therapy because it's "good enough" as a monotherapy to ditch the uncomfortable mask.

What's needed is a study that tests 1) whether GLP-1s work as a monotherapy, and 2) whether CPAP offer additional benefits beyond GLP-1s. In June 2024, Surmount-OSA clinical trials were published in the New England Journal of Medicine addressing these questions. The 52-week study evaluated two groups: 1) patients with moderate to severe OSA and obesity and were not on CPAP therapy, and 2) patients who were on CPAP therapy and used it alongside tirzepatide. Group 1 achieved a 47.7% reduction in their Apnea-Hypopnea Index (AHI). Group 2 achieved a 56.2% reduction.

In other words, GLP-1 monotherapy is clinically inferior to GLP-1 therapy paired with CPAP. Why? GLP-1s successfully reduce adiposity and so reduce patients' AHI, but they do not address the physical mechanics of airway collapse.

Next, we need a study that measures whether people on GLP-1s tend to continue using CPAP, or whether they abandon CPAP therapy. ResMed commissioned a longitudinal IQVIA claims study tracking patients from July 2021 through September 2025 with approximately 2 million patients with OSA. OSA patients prescribed a GLP-1 showed a CPAP initiation rate 10.8% higher. Thus, GLP-1s do not deter CPAP therapy - they accelerate adoption. Further, 1-year resupply adherence was 6.2% higher than the control group, and for 3 years resupply adherence remained high at 3.1%. The idea is that GLP-1s reduce mask pressures, making the CPAP treatment more tolerable, so people stick with it because it makes them feel better.

Business Quality & Economic Moat

RMD operates in two segments: 1) Sleep and Breathing Health, and 2) Residential Care Software.

Sleep and Breathing Health

ResMed has a Razor/Razor-Blade model, where the company sells the equipment, and then resupplies the cushions and mask interfaces, which need to be replaced every 30 to 90 days, with 70%+ margins. This creates an annuity cash stream with high lifetime customer value.

Software

ResMed patients use platforms like AirView (for the healthcare provider) and myAir (for the patient) that are cloud-based tools used to track and manage OSA, and are tied to reimbursements. This introduces switching costs, as it is difficult to switch hardware vendors to a new system.

Duopoly / New Asymmetry

Prior to 2021, the market was split between ResMed and Philips, but in 2021 Philips initiated a massive recall of its devices. ResMed captured approximately 80%-90% of those new CPAP placements. Today, ResMed is the industry standard.

Margins and ROIC

While device hardware gross margins are in the 50% range, masks and software are closer to 70%-80%, and ResMed has consolidated blended gross margins of 61%, which we think is strong. With respect to ROIC:

GAAP EBIT = \$1.9 bn, with an effective tax rate of 20.6% = \$1.5bn NOPAT

Total Invested Capital = \$6.6bn equity + \$0.8bn debt & leases - \$1.5bn cash = \$5.9bn Invested Capital.

NOPAT / Invested Capital = ~25%.

We think this places ResMed in the top 10% of all publicly traded companies meeting our liquidity requirements. With a WACC in the 7.5%-8.5% range, ResMed produces profits above the opportunity cost of the capital required to run it, and demonstrates ResMed's economic moat.

Capital Light Business Model

A Capital Light business model generates high ROIC because it does not require heavy capex. In a capital intensive business, capex might consume half or more of operating cash flow. Meanwhile, consider ResMed's profile:

ResMed's 2026 Capex = \$0.16bn

ResMed's Cash from Operations = \$1.8bn

Capex / Operating Cash Flow = 8.7%

Over 91% of operating cash flow is converted to free cash flow, giving the company flexibility for share repurchases and dividends.

Return of Shareholder Capital

During 2026, the company repurchased \$700mm of stock (133% YoY increase), and paid \$350mm of dividends (12.5% increase). With total FCF of \$1.65bn, the company returned 64% of its free cash flow to shareholders.

Valuation

Today, ResMed trades at \$228/sh and its valuation is as follows:

Market Cap: \$33 bn

Plus: Debt of \$659mm + Leases \$167mm = \$826mm

Less: Cash of \$1.5bn

Enterprise Value = \$32.3bn

GAAP EBIT = \$1.9bn + adjustments of ~\$140mm (intangibles, safety recall, restructuring) = Adjusted EBIT of \$2bn

EV/EBIT = ~16x

Bear & Bull Cases

The Bear Case assumes \$1.88bn EBIT (0% 2-year CAGR, margin erosion), and a target EV/EBIT of 14x, for a target share price of \$175 (-25% downside).

The Bull Case assumes \$2.3bn EBIT (10% CAGR), with a target EV/EBIT of 22x, and target share price of \$355 (51% upside).

Summary

We think ResMed presents a reasonable long opportunity as it generates a 25% ROIC and trades at a 16x EV/EBIT multiple, which is cheap for such a high-quality business. The short thesis fails to recognize that while GLP-1 weight loss makes masks easier to tolerate, it does not cure physical airway collapse. The company's high 91% FCF conversion demonstrates you are being paid well to look past the short narrative.

I hold a position with the issuer such as employment, directorship, or consultancy.

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Catalyst

- High-single digit growth in the Masks & Accessories segment
- Payers mandate that newly diagnosed OSA patients must try and fail CPAP before using GLP-1s
- Investors pricing in a return to a 50/50 duopoly see ResMed's market share as permanent

Messages

Subject	Other overhangs
Entry	09/09/2026 03:38 PM
Member	zzz007

Do you have a view on Apnimed's new product which has a PDUFA date in February? As an oral 1/day - compliance is likely higher - and at a minimum may be tried ahead of CPAP. Secondly - do you think if Phillips gets back to the market (estimates are sometime next year) - that it could also slow RMD's growth on the margin? Third - any concerns on payor mix changes as an incremental headwind (e.g., lower exchange coverage, higher deductibles - that have been called out by Orthopedic companies)?

No question it is a high quality business - I'm just not sure when some of these overhangs will be disproven